



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ماجده رجب محمد عبد المنعم

العنوان : البساتين

الحي / المدينة : الزيتون

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 19-09-2026

رقم البطاقة : 26712210103404 رقم التليفون : 01007599166

العمر (تاريخ الميلاد) : 21-12-1967 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: التوقيع:
الاسم: د/ندى حسن الوظيفة: التوقيع:
الاسم: د/نرمين رمضان الوظيفة: التوقيع:
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /..... /د. /..... /د. /..... /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: ماجده رجب محمد عبد المنعم National ID: 26712210103404	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



DMS
مدعى



Patient Name	ماجده رجب محمد	Case No.	344067
Sex/Age	Female 58 Y	Patient No.	25171
Registered Date	25/7/2026 04:10:11 PM	Referred By Dr.	Surgery
Validation Date	16/8/2026 10:29:12 AM	Branch	Gustave Roussy
Reporting Date	16/8/2026 12:41:09 PM	Pathology No.	2340/26

Pathology Report

Clinical Data: Right breast cancer, post-NACT.

Nature of the specimen: Right modified radical mastectomy.

Gross Examination:

- * Right modified radical mastectomy specimen measured 27x17x10 cm, with covering skin ellipse measured 25x15 cm. Nipple & areola looked unremarkable grossly.
- * Serialing showed a tumor bed site 3.0x2.5x2.0 cm surrounding by grey, white rubbery area, located 6 cm from the deep margin.
- * Axillary fat entangling 11 lymph nodes, the largest measured 3.5x3.0x3.0 cm.

* Block designation:

* 17 Blocks:

1:6- G/W area., 7:9- Mass., 10:12- G/W area., 13- Deep margin., 14- N/A., 15:17- ALNs.

- Gross was done by; Dr. Aya Ezzat.

Microscopic examination:

- **Histological type:** No residual viable remnants of the previously diagnosed invasive tumor.
- **Tumor Cellularity:** 0%.
- **Ductal carcinoma in situ:** Absent.
- **Angioinvasion:** Absent.
- **Perineural invasion:** Absent.
- **Nipple and areola:** Free.
- **Lymph node involvement:** Eight lymph nodes free of metastatic deposits (0/8).
- **Resection Margins:** Free deep margin.
- **Histologic response to therapy:** Pathological complete response.

Diagnosis:

Right Breast Cancer (Post-NACT); Right Modified Radical Mastectomy:

- No residual viable remnants of the previously diagnosed invasive tumor. (Cellularity 0%).
- Ductal carcinoma in situ: Absent.
- Nipple and areola: Free.
- Deep margin: Free deep margin.



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
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- Axillary lymph nodes: Eight lymph nodes free of metastatic deposits (0/8).
 - Histologic response to therapy: Pathological complete response.
- Stage: ypT0, ypN0.

Signature
Prof. Dr. Rana El Saeed.



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 		مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية		08/30/2026
MDT FORM				
Name: Mageda رجب Mohamoud Abd Elmenaam		MRN: 8114		
Age: 59 Y		ID: 26712210103404		
Menopausal Sta: post		Phone: 01003911732		
NRS SCORE:		Marital Status:		
BMI:		Family History:		
CO Morbidity: HTN DM IHD		Reason Of MDT: post op		

Clinical Data:

BX: IDC
 ER: 0 PR: 0 HER2: 3 KI67: 70

Sonomammography:**Mets Workup:****History:**

rt breast - multicentric pathological process with ipsilateral three levels 1 & 2 axillary lymph nodes

LAST SESSION 20/4/2026

postm rt breast cancer ct4b n1 mo er0/8 pr0/8 her2 3+, received NACT TCHP x6 (weekly) and 7 phesgo, last on 21/4 and was planned for MRM

Right Breast Cancer (Post-NACT); Right Modified Radical Mastectomy:

- No residual viable remnants of the previously diagnosed invasive tumor. (Cellularity 0%).
- Ductal carcinoma in situ: Absent.
- Nipple and areola: Free.
- Deep margin: Free deep margin.

Axillary lymphnodes: Eight lymph nodes free of metastatic deposits (0/8).

- Histologic response to therapy: Pathological complete response.

Stage: ypT0, ypN0.

for possibility of Adj tt

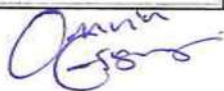
Recommendations:**MDT Outcome & Plan**

A case of Her2+ cT4b N1 disease, in PCR after NACT TCHP, but declining EF, For:

1. PORT to RT CW & PCLNs
2. Cardiological consultation & ensure use of BB & ACEI in her tt, then a new Echo, if EF>50%, resume adjuvant Phesgo (total 1 yr)...otherwise, we may omit adjuvant anti-her2 tt
3. Regular follow-up

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. شيماء لاشين	د. لمنية عيسوي				



05/11/2026

MDT FORM

Name:	Mageda رجب Mohamoud Abd Elmenaam	MRN:	8114
Age:	58 Y	ID:	26712210103404
Menopausal Status:		Phone:	01003911732
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	HTN DM IHD	Reason Of MDT:	POST NACT

Clinical Data:

BX: IDC
ER: 0 PR: 0 HER2: 3 KI67: 70

R

L

Sonomammography:

rt breast - multicentric pathological process with ipsilateral three levels 1 & 2 axillary lymph nodes

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postm rt breast cancer ct4b n1 m1 er0/8 pr0/8 her2 3+, received NACT TCHP x6 (weekly) and 7 phesgo, last on 21/4 and was planned for MRM

Mets Workup:

FREE

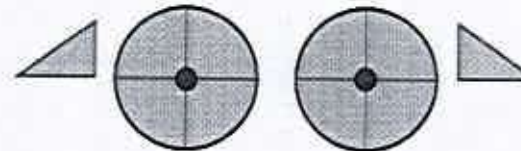
History:

Recommendations:

FOR RT MRM

MDT Outcome & Plan

RT MRM



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ محمد مصطفى	د/ محمد حسن				

MDT FORM

26/10/2025 G

Name	فايزة عبد الحليم	MRN	2662/2025
Age	58	ID	9251111111
Menopausal status	Postmenopausal	Phone	01003911732
NRS SCORE		Marital status	
BMI		Family history	-ve
CO morbidity	Cardiac (on ventral) (Atrial), HTN, DM	REASON OF MDT	NACT

Clinical Data

BX:

ER:

0/8

PR:

0/8

HER 2:

34

Ki67:

60-70%

Sonomammography: (29.8.2025)

Rt - Multicentric masses, largest: 1.0x4.6x.7cm
9-10:00, 1.3cm & 0.9cm

Mets Workup

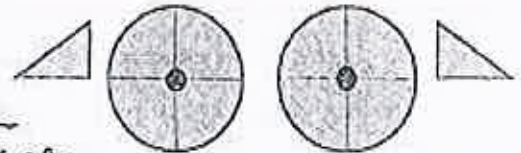
CTCAP: 1.0x0.6cm

Rt Axillary (4: 3.8x3cm)

Bone scan: -ve

R

L



TNM

2 2

(Multicentric)

Recommendations:

58 y/o d.f., PSI, EF 55%

Rt Locally advanced breast cancer CTq N, Mo, HER2 enriched.
y/o: 40-50%
Coastal

MDT OUTCOME & PLAN

for NeAdjuv TCHP X 6 cycles → MRM.

SIGNATURE

MEDICAL ONCOLOGY

SURGERY

RADIOLOGY

PATHOLOGY

Nutrition

Psychiatric

[Signature]

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[Signature]

Name : الأستاذ / ماجده رجب محمد عبد المليم
No. : 26712210103404
Age : 2770/25
Sex : Female
Age : 57 Y
Referring Doc :

Request No. : 13007519
Request Date : 08/09/2025
Verification Date : 15/09/2025
Financial Category : جوفات روسي - مياكر
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Right breast mass, Tru cut needle (TCNB) biopsy:
Invasive duct carcinoma, Grade 3 with focal necrosis .

IPT:

- * ER: Negative (score 0/8).
- * PR: Negative (score 0/8).
- * HER2: Positive (score 3+).
- * Ki67: 60-70 %
- * Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof Dr. Maha Guimei
Dr. Heba El-shafeey

Verified by:
Dr. Heba Hasan

Printing Date : 18/09/2025 09:26:30

Page 2 of 3

Printing By : Mrs.Mayada Hegazy



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Patient Name: Magda Ragab Mohamed Abdel Mon'em	Date: 28/08/2025
Exam: ultrasound BREAST (BOTH SIDE)	Age: 57 Year(s)
Medical ID: 10471	Pat ID: 111182

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: 45 years old female with positive family history (maternal side aunt), complaining of left breast lump of a month duration.

Findings:

Right breast:

- Scattered fibroglandular densities (ACR b)
- UOQ, UIQ and LOQ, through-out the whole breast thickness, global asymmetry with overlying suspicious microcalcifications and surrounding distortion measuring about 13.5 x 12 cm.
- Ultrasound demonstrated UOQ, UIQ and LOQ dilated ducts reaching 5 mm in caliber with soft tissue echogenicity, microcalcifications and internal vascularity within, mounting to mass formation at the LOQ measuring 45 x 17 mm with overlying focal skin thickening reaching 3.5 mm.
- These changes are seen involving the nipple
- Two irregular spiculated heterogenous hypoechoic solid mass lesion with surrounding echogenic halo, posterior acoustic shadowing and mild internal vascularity on color Doppler study seen at 9-10 o'clock measuring 13 x 13 mm and 8 x 8 mm
- Two level 1 pathological axillary LNs are noted the largest measured 38 x 22 mm with 20 mm cortical thickness and one detected at level 2 measuring 14 x 13 mm.

Left breast:

- Scattered fibroglandular densities, (ACR b)
- No spiculations, masses asymmetry, distortion or suspicious microcalcifications
- Benign macrocalcifications
- Ultrasound demonstrated retro-areolar duct ectasia reaching 5mm with clear content
- Normal skin/nipple/areola complex
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 5): multicentric pathological process with ipsilateral three levels 1 & 2 axillary lymph nodes.

Left breast (BIRADS 2): Benign calcifications and duct ectasia.

Recommendation:-

For US guided Tru-cut biopsy and histopathological correlation.

Best Regards,
DR. Amal Hegazy, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendation	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0%
BIRADS 2: Benign findings, routine screening	0%
BIRADS 3: Probably benign, for close follow up.	2%
BIRADS 4: Probably malignant, for biopsy.	50%
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	99-100%
BIRADS 6: Pathologically proven malignant	

Signature Dr. Amal Hegazy
PM 2:36 2025/08/28

Patient Name: Magda Ragab Mohamed Abdel Mon'em	Date : 05/10/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 58 Year(s)
Medical ID: 2662/2025	Pat. ID: 111182

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported case of right breast carcinoma.

Chest:

Patent central air ways with well inflated both lungs.

Mosaic attenuation of both lower lobes ,likely vascular/congestion.

Right lower lobar apical subpleural bleb is noted.

No parenchymal nodules, cavitary lesions, consolidative patches or bronchiectatic changes.

No CT evidence of hilar or mediastinal lymph node enlargement.

No pleural or pericardial collections seen.

Cardiomegaly .

Right breast LOQ lesion ,for sonomammographic assessment.

Right axillary pathological lymphadenopathies, reaching 38x30 mm in diameter.

Abdomen and pelvis:

The liver is mildly enlarged in size showing homogenous reduced CT density with no focal lesions.

Patent and homogenously enhancing portal vein.

No intra or extra hepatic biliary radicles dilatation.

The spleen is seen mildly enlarged with no focal lesions.

Right renal marginally calcified cyst is seen measuring 39x37 mm .

Cholecystectomy clips is noted.

CT appearance of the pancreas, suprarenal glands, both kidneys, apart from right renal cortical cysts, aorta and IVC.

Regular filling of the urinary bladder showing no stones, masses or diverticulae.

Bulky uterus with isodense lesions.

No retro-crural, para-aortic or pelvic lymphadenopathy.

No ascites.

L5 1st degree degenerative spondylolisthesis.

INION:

Clinically reported case of right breast carcinoma showing the above detailed changes with no CT evidence of hepatic or pulmonary metastases.

Mild fatty hepatomegaly.

Mild splenomegaly.

Cardiomegaly, for echocardiographic assessment.

Bulky uterus? fibroids, for sonographic assessment.

Dr.Sara Mahmoud, MD

Sara Ali

Name : Magda Ragab Mohammed Abdul-Moneim

Date : 29-09-2025 01:39

Branch : Dokki

Medical No. : 213800

Ref.Physician Prof : Gostaf Roseh Hospital

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History

- A 57-year-old lady, with history of right breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

- No abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Name: ماجدة رجب محمد

Date: October 14, 2025

AGE: 57

Echocardiographic Report

LVEDD	49	35 - 57	LA	36	23 - 40
LVESD	34		AO	24	23 - 40
SWT	13	6 - 11	RV	2.1	1.6 - 2.6
PWT	8.4	06 - 11	EF	55	
FS %	29	25 - 45			

Comment

- Normal aortic root and left atrium dimensions.
- Fair left ventricular systolic function with EF 55% by M-mode and visual estimation
- Normal internal wall dimensions.
- LVH
- Dyskinesia in anterior Septum
- Mitral inflow Pattern show Diastolic dysfunction grade II.
- Normal right atrium and Right ventricle dimensions and Preserved RV Function.
- Normal Mitral valve morphology with trivial MR
- Normal Aortic valve morphology with Trivial AR
- Normal Tricuspid valve morphology with trivial TR with normal RVSP
- Normal Pulmonary valve morphology with normal flow.
- No thrombi or vegetations could be detected in LV.
- Intact Interatrial and interventricular septi.
- Normal pericardium, no evidence of pericardial effusion.

Conclusions: Echo-Doppler study suggestive

- ✦ Fair LV systolic function.
- ✦ Diastolic dysfunction grade II
- ✦ LVH.

Much Oblged
Dr. Hany said
MSc Cardiovascular medicine

8/30/26, 3:57 PM

code:02 -- Name_A MDT -- Name_E MDT

 		مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية		الصحة المصرية مصر	
08/30/2026					
MDT FORM					
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Sonomammography:

Mets Workup:

History:

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- Deep margin: Free deep margin.

Axillary lymph nodes: Eight lymph nodes free of metastatic deposits (0/8).

- Histologic response to therapy: Pathological complete response.

Stage: ypT0, ypN0.

for possibility of Adj tt

Recommendations:

MDT Outcome & Plan

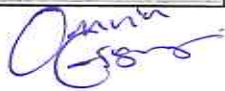
A case of Her2+ cT4b N1 disease, in PCR after NACT TCHP, but declining EF,

For:

1. PORT to RT CW & PCLNs
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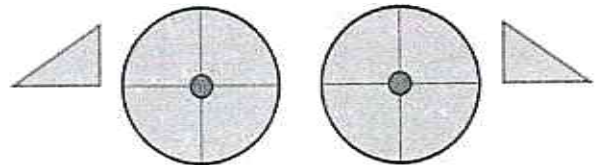
SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. شيماء لاشين	د. لمنية عيسوي				



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